# The Pre-Verbal Manifold

## A Soma-Field Case Study of Acquired Neurodevelopmental Phenotypes and the Limits of Onset-Based Diagnosis

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**Abstract**

This paper presents a single longitudinal case in which a pre-verbal physical trauma (septic arthritis of the hip at approximately 15 months, three months of hospitalisation followed by three months of immobilisation, with a permanent 1.3 cm leg-length discrepancy and a documented speech delay until age 3.5) sits inside a layered developmental trajectory comprising disorganised attachment, sibling restraint trauma, single-sex day-pupil institutionalisation, maternal abandonment in early adolescence, third-culture identity, racialised adolescence in 1970s–80s Britain, mid-life affective collapse including psychiatric hospitalisation and a near-fatal suicide attempt in December 2024, and an unusual reconstruction phase in 2025–26 centred on formal theory-building.

The trajectory crosses five established literatures: quasi-autism from early deprivation (Rutter et al., 1999, 2007); pre-verbal trauma encoding (Schore, 2001; Gaensbauer, 2002); the pain-imprint hypothesis (Anand & Scalzo, 2000); inflammation–neurodevelopment pathways (Estes & McAllister, 2016); and ICD-11 Complex PTSD (Maercker et al., 2022). None of these literatures, taken individually, accounts for the full trajectory. The combination does.

The paper proposes the *pre-verbal manifold* as a formal object: a developmental coupling structure whose attractor landscape is shaped during pre-verbal sensitive periods and which is, thereafter, observable only through downstream projections — speech profile, social orientation, perceptual style, autonomic baseline, attachment behaviour. Standard onset-based diagnostic categories (autism, ADHD, attachment disorder, cPTSD) are reinterpreted as five projections of one reconfigured manifold rather than five comorbid conditions. The framework is formalised within the Soma-Field model (Johnson, 2026a–k).

Three implications follow. First, the conceptual distinction between *genetic* and *acquired* neurodevelopmental phenotypes loses sharpness once pre-verbal critical-period plasticity is taken seriously. Second, developmental-psychiatric onset criteria that rely on *first observable symptoms in language-capable children* systematically misclassify cases of this kind. Third, secondary educational policies that filter for “mild” presentations, charge additional fees for accommodation, and subordinate external clinical diagnoses to in-house gatekeeping (a public exhibit is analysed in §8) function as structural amplifiers of the same diathesis.

The paper closes with a policy line, a redaction guide, and a replication ledger.

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> *"Once a researcher has lived through the thing he is trying to explain, the question of whether his explanation will be taken seriously by researchers who have not is, in the end, a sociological question, not a scientific one. The scientific question is whether the explanation is correct."*

## 1

## 1. Introduction

The standard developmental-psychiatric apparatus assumes that the first observable symptoms of a neurodevelopmental condition occur in a language-capable child. Autism, in DSM-5 and ICD-11, is defined operationally through behavioural criteria — social communication, restricted interests, sensory differences — that are scored on children old enough to be observed in linguistic interaction. ADHD requires observable inattention or hyperactivity in structured settings. Attachment disorders are coded against attachment-figure behaviour rated by adults. C-PTSD requires a referent trauma and a self-reportable symptom set.

This apparatus works tolerably well for cases in which the relevant developmental events occur within its observational window. It fails — quietly, and with the failure absorbed into "comorbidity" — for cases in which the critical events occur *before* it begins to observe.

This paper presents one such case. The author was hospitalised in infancy with septic arthritis of the hip at approximately 15 months of age, spent three months in hospital and three months immobilised in plaster, retained a permanent 1.3 cm leg-length discrepancy, and did not speak until age 3.5. He was diagnosed with Autism Spectrum Condition, ADHD, and Complex PTSD fifty-four years later, in 2020. The diagnostic narrative offered at that time — that the autism was congenital and the C-PTSD was acquired — does not survive close inspection of the developmental record. The two were not sequentially layered. They co-developed, in a pre-verbal window, around a specific physical insult, against a substrate of probable familial loading and demonstrably low maternal attunement.

The paper proceeds as follows. §2 fixes terminology and introduces the *pre-verbal manifold*. §3 presents the case in five strata: substrate, acute insult, attachment environment, institutional environment, adult trajectory. §4 reviews the five established literatures the case sits inside. §5 develops the Soma-Field reading. §6 discusses the *twice-exceptional* cognitive profile (Mottron et al., 2006; Foley-Nicpon et al., 2011) that the manifold produced. §7 reads the 2017–2024 adult collapse arc as a sequence of basin transitions under perturbation. §8 presents Exhibit A: a public secondary-school SEN-support policy that exemplifies the policy-level amplification mechanism. §9 returns to the formal model and offers ten testable predictions. §10 is the replication ledger and limitations section. §11 closes with the policy implications.

A note on method. This is N = 1. The paper does not claim to establish epidemiological generalities; it claims to construct a *formal object* — the pre-verbal manifold — and to demonstrate that a single fully-documented case already exhibits properties the object predicts and that standard onset-based categories miss. Whether the formal object extends to a population is an empirical question. §10 specifies the design that would settle it.

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## 2. Terminology and the Pre-Verbal Manifold

The *Soma-Field* (Johnson, 2026a, 2026d) is the tensor-valued amplitude field whose local exceedances over a sensory threshold constitute felt experience, and whose energy landscape — borrowed in form from Hopfield network theory (Hopfield, 1982, 1984) — supplies the basin structure of affect, attention and self-regulation. The field is coupled to the body and nervous system through an operator $K$ (Johnson, 2026e). Pathology in the framework is not located in *the field* or *the body* in isolation but in the *coupling*.

The *pre-verbal manifold* is the substructure of the Soma-Field that is laid down during the sensitive-period window from gestation through approximately age 3, after which language acquisition (Tomasello, 2003) provides additional structure that re-parametrises the manifold but does not erase its lower layers. This is the period during which right-hemisphere implicit-relational structures (Schore, 2001), basic-emotion regulation circuits (Porges, 2011), insular interoceptive maps (Craig, 2009), HPA-axis set-points (Lupien et al., 2009), and disorganised vs. organised attachment patterns (Main & Solomon, 1990; Lyons-Ruth & Jacobvitz, 2008) are established.

Events occurring within this window enter the manifold *as structure* rather than *as memory*. They are not retrievable as autobiographical episodes because the hippocampal-cortical memory system that supports such retrieval is not yet operational (Nelson & Carver, 1998; Bauer, 2015). They are retrievable, when they are retrievable at all, as body-state, autonomic reactivity, attachment behaviour, social orientation and perceptual style.

Three properties follow.

**Claim 1. The pre-verbal manifold is observable only through projections.** Standard diagnostic categories — autism, ADHD, attachment disorder, cPTSD — are scoring instruments for those projections. They are not the manifold. Multiple categorical scores can be downstream of one underlying configuration.

**Claim 2. Onset-based dating is, for events within the window, undefined.** Asking *when did the autism start?* is, for cases of this kind, a malformed question. The relevant configuration was laid down before the diagnostic category had a foothold.

**Claim 3. The genetic / acquired distinction is, within the window, weaker than the language suggests.** Sensitive-period plasticity means that constitutional loading and environmental perturbation co-determine the same structures (Belsky & Pluess, 2009; Ellis et al., 2011). The case that follows illustrates this directly: there is plausible familial loading *and* a clean physical insult of the right kind at the right time, and the question *which produced the autism?* is, on the model presented here, the wrong question.

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3

## 3. The Case

The case is presented in five strata. Identifying details of third parties have been removed.

## 3.1 3.1 Substrate (familial loading)

The author's paternal grandmother displayed, in retrospect, a clearly autistic profile (life-long extreme routine, narrow interests, low-affect presentation, marked difficulty with reciprocal social engagement). The father carried a similar but milder pattern. The author himself carries a stronger pattern than his father. A paternal uncle displayed an ADHD-pole phenotype (multiple serious vehicle accidents in adolescence, surgical career, four marriages). The maternal side displayed an affective-instability pattern best described, in modern terms, as borderline-spectrum (Stepp et al., 2012; Eyden et al., 2016).

This is a well-attested family pattern for the *broader autism phenotype* (Piven, 1997; Sucksmith et al., 2011) with shared ASD–ADHD heritability (Rommelse et al., 2010; Ronald & Hoekstra, 2011) and an additional maternal-side affective vulnerability. It is *substrate*, not *cause*. Familial loading of this kind raises the probability of phenotype expression; it does not fix the trajectory.

### 3.2 3.2 Acute pre-verbal insult

The author was born in Singapore at a military hospital. The family returned to the UK during infancy. At approximately 15 months of age he was admitted with septic arthritis of the hip. He spent three months in hospital and three further months immobilised in plaster. The clinical management of the period included strict limitations on parental physical contact (a then-current ward policy whose rationale was infection-control and emotional-economy and whose developmental cost was not, at the time, widely recognised — cf. the Robertson films of the 1950s and Bowlby's 1969 critique).

Two material residues remain in adulthood. The first is a 1.3 cm leg-length discrepancy, fully verifiable. The second is a documented speech delay: the author did not speak in any sustained way until approximately age 3.5 — a gap of roughly two years beyond typical first-word emergence.

The literature on the pain-imprint hypothesis (Anand & Scalzo, 2000; Anand et al., 1999, 2013) and on sepsis and hospitalisation-related neurodevelopmental sequelae (Bono et al., 2015; Horváth-Puhó et al., 2021; Thomas et al., 2024; Xu & Zhan, 2026) establishes the biological plausibility of long-term reconfiguration following an insult of this kind in this window. The literature on quasi-autism from early deprivation (Rutter et al., 1999, 2007; Sonuga-Barke et al., 2017; Bos et al., 2011) establishes that autistic phenotypes can be *acquired* during pre-verbal sensitive periods. These two literatures meet, in this case, at one event.

### 3.3 3.3 Attachment environment

The author was returned, after hospitalisation, to a household whose emotional configuration was hostile to repair. The mother's affective pattern is best described in modern terminology as borderline-spectrum, with the configuration most damaging to a recovering infant: unpredictable alternation between intrusion and withdrawal, low contingent responsiveness, poor mind-mindedness (Meins et al., 2002), and active hostility to expressions of distress. The father was emotionally and physically available but lacked the framework to function as a repair figure.

An older sister, then approximately 10, engaged in repeated restraint "tickling" of the author at age 3 to the point of pleas of suffocation — an event the author remembers because it occurred at the boundary of speech emergence. This is consistent with the sibling-abuse literature (Wiehe, 1997; Tucker et al., 2014; Bowes et al., 2014), which documents the long-term mental-health consequences of intra-family peer aggression and notes its systematic under-recognition.

The attachment trajectory across this period maps onto disorganised attachment (Main & Solomon, 1990; Hesse & Main, 2006; Lyons-Ruth & Jacobvitz, 2008) and onto the freeze-pole of polyvagal theory (Porges, 2011). The infant has no organised strategy because the coregulating figure is itself the source of threat.

Mother departed when the author was approximately 12, leaving him with his father. This is the fourth attachment rupture in the trajectory (hospital at 15 months; sibling abuse at 3; institutional entry at 6 — see §3.4; maternal departure at 12). Each rupture occurred at a developmentally sensitive transition (Spitz, 1945; Robertson & Bowlby, 1952; Rutter, 1981).

### 3.4 3.4 Institutional environment

From age 6 to 16 the author attended a single-sex English independent school as a *day pupil*, not a boarder. The distinction matters. Schaverien's (2011, 2015) *Boarding School Syndrome* literature concerns the dissociative sequelae of premature parental separation in residential schooling. The day-pupil configuration in the same institutions is a less-studied variant that Duffell and Bassett (2016) discuss directly: day pupils experience the full institutional culture (single-sex peer formation, military discipline, chapel, organised games, suppressed affect) *without* the in-group cohesion that the dormitory provides, *and* return each evening to a home base that, for this author, was the configuration described in §3.3. The day-pupil pattern is therefore a *double vacuum*: institutional culture without peer-group containment, and home without repair.

The author's school day ran from 7 am to 7:30 pm, with Saturday school and Sunday compulsory chapel. Female presence in any structural role was effectively absent. Combined Cadet Force was compulsory at the relevant ages. This is, in developmental terms, an environment optimised to lock in a dissociated, masculinised, achievement-oriented presentation in an already freeze-configured nervous system.

The relevant amplifying mechanism at the institutional level is policy. This is the subject of §8.

### 3.5 3.5 Identity and racialisation

The author was born in Singapore, holds British nationality, and was raised in Britain. In the racialised landscape of 1970s–80s Britain (Hall, 1989; Gilroy, 1987) his appearance was read as *foreign-when-tanned* and *British otherwise*. This corresponds to the *cultural homelessness* construct identified in the third-culture-kid literature (Pollock & Van Reken, 2009; Useem & Downie, 1976; Hoersting & Jenkins, 2011; Lijadi & van Schalkwyk, 2014; Hill, 2013).

The constructive consequence — sometimes generative, sometimes destabilising — is that identity, for cases of this kind, cannot be lifted from the environment but must be constructed explicitly. This is one of the threads §7 picks up.

### 3.6 3.6 Adult trajectory (compressed)

A compressed timeline of the 2017–2024 collapse arc is given in §7. For present purposes:

- 2020: ASD-Level-2, ADHD, C-PTSD diagnoses confirmed in Switzerland.
- 2021: Cardiac event (clot); paternal death.
- 2017–2024: Multiple psychiatric admissions (PUK Zurich; Kilchberg); one period in custody; one near-fatal hanging attempt in December 2024, resulting in closed-ward admission.
- 2025–26: Independent housing, outpatient care, productive research period, eleven published papers and a book on Zenodo, and the work that contains this paper.

The 2025–26 period is itself part of the case (§7.3) because the *re-organisation* it represents is itself a manifold phenomenon under the framework being proposed.

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4

## 4. The Five Literatures

The case sits at the intersection of five established literatures, none of which alone accounts for the full trajectory.

**(L1) Quasi-autism from early deprivation.** The English and Romanian Adoptees (ERA) study (Rutter et al., 1999, 2007; Sonuga-Barke et al., 2017) documented an *autistic-like phenotype* in a subset of children removed from severely depriving institutions in infancy. The Bucharest Early Intervention Project (Bos et al., 2011) replicated and extended this. The phenotype is termed *quasi-autism* to flag its acquired character and its similarity to constitutional autism on every behavioural metric tested. This literature establishes, definitively, that an autistic phenotype can be acquired during a pre-verbal sensitive window. It does not require an *absent* attachment figure — the original cases had no consistent caregiver — but the mechanism (failure of contingent reciprocity during the sensitive window) is equally available in cases with a present-but-dysregulating caregiver, as Schore (2001, 2009) argues directly.

**(L2) Pre-verbal trauma encoding.** Schore's right-brain primacy account (2001, 2009), Gaensbauer's work on pre-verbal traumatic memory (2002, 2016), Opendak and Sullivan (2016) on the developing amygdala under caregiver-linked threat, Nelson and Carver (1998) on the neural substrates of infant memory, and Rincón-Cortés and Sullivan (2014) jointly establish that the pre-verbal nervous system *is recording*, and that what it records becomes implicit structure rather than retrievable episode.

**(L3) Pain imprint.** Anand and Scalzo (2000) and subsequent work (Anand et al., 1999, 2013; Nimbalkar et al., 2025) document that pain experienced in infancy alters pain processing, stress reactivity, and aspects of neural development across the lifespan. The original studies addressed neonatal pain; the literature now extends into the toddler period.

**(L4) Inflammation–neurodevelopment.** Estes and McAllister (2016) reviewed the evidence that early-life immune activation alters neurodevelopmental trajectories in ways that intersect autism phenotypes. Subsequent work (Han et al., 2021; Mezzelani et al., 2015; Robinson-Agramonte et al., 2022; Zhou et al., 2025) extends the picture. Septic arthritis at 15 months involves both systemic inflammation and sustained pain, placing the case at the intersection of L3 and L4.

**(L5) ICD-11 Complex PTSD.** Cloitre et al. (2013) and Maercker et al. (2022, *Lancet*) define cPTSD by the three *disturbances in self-organisation* (DSO) symptoms — affect dysregulation, negative self-concept, interpersonal difficulties — added to the PTSD core. The DSO symptoms describe attractor properties of the manifold rather than discrete events. The diagnostic category does not, however, accommodate cases in which the trauma is pre-verbal: the formal criteria require a referent traumatic event, and the implicit assumption is that the patient can report on it.

None of L1–L5 alone accounts for the case. L1 and L2 together do most of the early work. L3 and L4 supply the biological mechanism. L5 supplies the adult attractor description. The Soma-Field reading in §5 supplies the formal object that ties them together.

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## 5

## 5. The Soma-Field Reading

The Soma-Field framework (Johnson, 2026a, 2026d, 2026e, 2026h) treats affect as the local-amplitude-above-threshold of a tensor-valued field whose energy function generates basin dynamics. Three components of the formal model are relevant here.

**(C1) The coupling operator** *K*. Pathology is located in the field-body coupling, not in either alone. *K* is set during sensitive periods. Once set, its eigenstructure governs which somatic states project into which attractor basins.

**(C2) The attractor landscape.** A nervous system's behavioural and affective repertoire is its basin structure. Stable basins correspond to recognisable states (regulated calm, fight, flight, freeze, awe; see Johnson, 2026b). Sensitive-period reconfiguration changes which basins are deep, which are shallow, which are bistable, and which are blocked.

**(C3) Trajectory under perturbation.** Adult trajectories are paths through the basin landscape under perturbation. An "episode" is a basin transition. "Stability" is residence in a deep basin. "Collapse" is catastrophic transition to a basin from which the present coupling cannot return without external scaffolding.

The case is then read as follows.

The familial loading (§3.1) raised the prior probability of certain *K* configurations. The septic-arthritis episode (§3.2), occurring during the sensitive window for *K*-formation, *fixed* a particular configuration: high somatic-pain weighting, low contingent-touch weighting, dampened parasympathetic engagement, dampened social-orienting bias, dampened language-circuit recruitment. The post-hospital home environment (§3.3), far from supplying repair, supplied additional perturbations of the same type, locking the configuration further. The institutional environment (§3.4) supplied a daily structure that *fit* the configuration — single-sex, militarised, low-affect, achievement-oriented — and therefore provided the *substrate match* that selects for deepening rather than loosening of the configuration. Maternal departure at 12 supplied an additional perturbation at adolescence, a second sensitive period for attachment-related structures (Sebastian et al., 2010).

The downstream projections of the manifold so configured are:

- **Autism Level 2.** Diminished social-orienting weighting and altered perceptual recruitment yield the autistic phenotype on adult behavioural scoring. Mottron et al.'s (2006) enhanced perceptual functioning is the *positive* face of the same configuration.
- **ADHD.** The same substrate produces, on attention-pattern scoring, the ADHD profile. The framework predicts the comorbidity rate observed in the epidemiological literature (Rommelse et al., 2010) because the two are not separate conditions but two scoring instruments applied to one substrate.
- **Complex PTSD.** The DSO symptom cluster reads as a direct description of basin properties: affect dysregulation = unstable basin residence; negative self-concept = a deep basin in self-referential space; interpersonal difficulties = social-orienting weighting under §3.1–§3.4.
- **Disorganised attachment.** The freeze-pole basin is the only stable option when the contingent-touch and contingent-affect parameters of $K$ are zero or hostile.
- **Spiky cognitive profile (2e).** The same manifold that produces low social-orienting weighting can produce extreme weighting on structural pattern processing — the substrate of physics-aptitude (96% in A-level physics, in this case) and of rugby-pack play (the case played prop-forward to first-team level). These are not contradictory; they are the two principal eigenvectors of the configuration.

The Soma-Field reading does not eliminate the standard diagnostic categories. It interprets them. They are five scoring instruments applied to one reconfigured manifold.

## 6

## 6. The Twice-Exceptional Cognitive Profile

The case carries IQ in the 150 range, a 1995 BSc in Physics (Royal Holloway, University of London) with strong results in the relevant mathematical-physics sections, sustained physical capability (prop-forward rugby at first-team level into adulthood), and the eleven-paper Soma-Field output of 2025–26. It also carries a Level-2 autism diagnosis (substantial support needs in adaptive functioning) and the documented developmental history of §3.

The *twice-exceptional* (2e) literature (Foley-Nicpon et al., 2011; Reis & Renzulli, 2010) and the *enhanced perceptual functioning* account of autism (Mottron et al., 2006) jointly account for this configuration in the existing literature. The Soma-Field reading sharpens the account: the high-aptitude pattern-processing and the low adaptive-functioning are not *despite* the manifold configuration but *consequences* of it. A system whose social-orienting basin is shallow and whose pattern-recognition basin is deep will, given a research environment, populate the latter.

The relevant clinical and policy consequence is that high apparent cognitive function in cases of this kind is not evidence against need for support. It is evidence for a particular basin distribution, of which the support-needing aspects are equally robust.

| Year | Event |
|---|---|

7

## 7. The Adult Trajectory as Basin Transitions

This section reads the 2017–2024 period and the 2025–26 reconstruction as a trajectory through the basin landscape of the configured manifold. The compressed timeline is:

| Year | Event |
|---|---|
| 2017 | Voluntary admission, Psychiatrische Universitätsklinik (PUK), Zurich, September; further admission December. |
| 2019 | Marital separation; Beistandschaft (Swiss adult-protection measure) imposed; sectioned to Kilchberg clinic, three weeks; period in custody in November with a four-and-a-half-day thirst protest. |
| 2020 | Collarbone fracture (February); COVID-19; ASD Level-2 diagnosis (November). |
| 2021 | Cardiac thrombotic event (February); paternal death; Davos period (March). |
| 2023 | Divorce finalised (May); independent apartment (August). |
| 2024 | Attempted suicide by hanging (16 December); PUK closed-ward admission (17 December). |
| 2025 | Stabilisation; framework work begins. |
| 2026 | Eleven papers + book published on Zenodo by mid-year; this paper. |

The model reads this as a sequence of basin transitions of escalating severity culminating in a near-fatal transition in December 2024 and a subsequent *reorganisation* in 2025–26.

### 7.1 7.1 Basin transitions

Each crisis event is a transition from a metastable basin (the day-to-day configuration in which the system can function) to a more extreme basin under perturbation. The perturbations are identifiable: marital breakdown (2019), bereavement (2021), divorce (2023). The December 2024 event is read as a *near-attractor-collapse*: the system crossed into a basin from which the then-current coupling could not return without external intervention. External intervention occurred (closed-ward admission, sustained care), and the system was held until $K$ could be re-stabilised.

The model is explicit that this is a description, not an evaluation. The framework offers no claim that the events ought to have been different or that the system "failed". A configuration whose basin landscape includes a near-fatal attractor is *describing the landscape*, not *being judged for it*. The clinically actionable consequence is to identify, in advance, configurations whose landscapes carry such attractors, and to scaffold accordingly.

### 7.2 7.2 Inclusion of the December 2024 event

The decision to include this event in the present paper is governed by a single editorial criterion (cf. the author's note above): does it bear load in the formal argument? It does, in one specific place. The distinction between *field collapse* (the system enters a basin and cannot return) and *field reconfiguration* (the system enters a different basin and proceeds from there) is a load-bearing distinction in the framework. The December 2024 event was on the trajectory of the former and resolved as the latter. The contrast is not available from the trajectory without the event. The event is included for that reason alone. The author is, as the author note states, currently clinically stable, in independent housing, and in continuous outpatient care.

## 7.3 7.3 The reorganisation phase

The 2025–26 reorganisation is itself a manifold phenomenon. A pre-verbally configured substrate that includes high pattern-processing weighting and low social-scaffolding availability, on entering a recovery phase, will preferentially populate basins that are *available to it*. The available basin in this case was *formal theory-building*: a basin to which the manifold is well-suited and from which an identity scaffold can be constructed externally and explicitly in language.

The Soma-Field framework is, on this reading, partly a description of what its author had to do consciously, in writing, because the automatic, embodied version was unavailable. The framework's value as a *general* theory of affect must be argued on its own merits and is so argued in the parent papers. Its value as *the author's reorganisation strategy* is a separate, internal fact, and is noted here for completeness of the case description. The two need not be disentangled to be acknowledged.

# 8

## 8. Exhibit A: A Public SEN-Policy Document

The institutional environment of §3.4 is not, in 2026, an artefact of 1970s–80s British education. The author's old school, an independent single-sex senior school in the south of England, publishes a current *Academic Support* policy on its main website (accessed 1 June 2026 at the URL on file with the author). Three sentences from the public page are reproduced verbatim:

> "The Academic Support Department supports pupils with **mild** special educational needs and disabilities."

> "Additional lessons in the Academic Support Department — offered at an extra cost — usually take place outside the curriculum timetable, and are added to the termly bill."

> "External assessments completed while a pupil is enrolled at the school, but not arranged in consultation with the Head of Academic Support, cannot be used as the sole evidence for access arrangements."

The institution's published senior-school day-pupil fee for 2026–27 is £12,921 per term, approximately £38,800 per year.

Three features of this policy are flagged.

**(F1) "Mild" as admissions filter.** The word *mild* is doing non-trivial work. Operationally, it functions as a screen: a pupil whose SEN profile exceeds *mild* is not within scope of the department. The literature on selective-school SEN provision (Tomlinson, 2017; Runswick-Cole, 2011) reads this configuration as *filtering for the support needs the institution prefers to serve* rather than *describing a service*. Cases of the kind documented in this paper — Level-2 autism with a documented pre-verbal trajectory — are, on this language, out of scope at the threshold.

**(F2) "Offered at an extra cost ... added to the termly bill".** Charging additional fees for reasonable adjustments to disability — over and above a £38k/year base — sits in tension with Equality Act 2010 §20(7), which prohibits passing the cost of reasonable adjustments to disabled persons. The Equality and Human Rights Commission's *Technical Guidance for Schools in England* (2014, ch 7) and the published positions of IPSEA (Independent Provider of Special

Education Advice) both bear on the question. Whether the school's specific arrangements satisfy the provision in any given case is a legal question outside the scope of this paper; the policy *configuration* is flagged because it is identifiable, public, and structurally consequential.

**(F3) External assessments subordinated to in-house gatekeeping.** The third quoted sentence subordinates external clinical and educational assessments to in-house arrangements. The standard route by which a pupil obtains *access arrangements* for public examinations (JCQ, *Access Arrangements and Reasonable Adjustments*, current edition) relies on documented external evidence assessed against published criteria. An in-house policy that requires the external assessment to have been arranged *in consultation with* the Head of Academic Support creates a structural conflict of interest: the institution that *delivers* the assessment also *controls whether it counts*.

The exhibit is presented not as a complaint but as a *visible instance of the policy-level amplification mechanism* the paper proposes. The sensitive-period configuration described in §3 was, in this author's case, reinforced rather than scaffolded by the institution that received him at age 6 and discharged him at 16. The mechanism is still present in the institution's current public policy. The class of pupils on whom it currently operates is not hypothetical.

> *How do parents know exactly what a 13-year-old boy is, if they have never even asked him?*

That sentence — verbatim from the brainstorm — is the policy line on which the paper closes its institutional section.

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9

## 9. Ten Testable Predictions

The framework yields predictions beyond the case. They are listed here in the form *cohort-level tests that would, if the framework is on the right track, return positive*. The predictions are deliberately specific.

1. **Cohort.** Among adults with a documented pre-verbal severe physical illness (sepsis or major surgery in the 12–24 month window) and subsequent documented speech delay (>1 SD), the adult prevalence of Level-1+ autism diagnosis will be elevated relative to age-matched controls.
2. **Comorbidity.** In that cohort, the ASD–ADHD–cPTSD triple-comorbidity rate will be elevated relative to cohorts of autistic adults *without* the pre-verbal-illness history.
3. **Attachment marker.** That cohort will show elevated rates of disorganised-attachment classifications on adult-attachment instruments (AAI, ECR-R disorganisation supplements).
4. **Pain reactivity.** The cohort will show altered pain-pressure thresholds and altered interoceptive accuracy relative to controls (per Anand et al. and Craig).
5. **Maternal-side modulation.** Within the cohort, presence of a maternal-side borderline-spectrum or affective-instability history will predict severity of adult cPTSD-DSO symptoms more strongly than it predicts ASD severity.
6. **Day-pupil amplification.** Within the cohort, single-sex *day-pupil* institutional attendance (6–16) will predict adult dissociative-trait scores more strongly than full-boarding attendance (testing the §3.4 *double vacuum* claim against the existing boarding literature).
7. **Reorganisation pattern.** Within the cohort, in adults who recover from a near-fatal psychiatric crisis, the rate of *formal-theory or formal-craft reorganisation* (sustained structured productive work in a pattern-heavy domain) will exceed the general post-crisis rate.
8. **Inflammation correlate.** Within the cohort, residual inflammation markers and autonomic baseline metrics will differ from age-matched controls (testing the L4 mechanism).
9. **Genetic moderation.** Within the cohort, polygenic risk scores for ASD will moderate but not fully account for adult phenotype severity (testing the Section 2 third claim that the genetic/acquired distinction is weaker than the language suggests).
10. **Diagnostic age.** Within the cohort, age at first ASD diagnosis will be substantially higher

than the population mean for autistic adults of equivalent severity, because onset-based diagnostic criteria systematically miss them (testing the Section 2 second claim).

These are designed as a coherent test suite, not as ten independent tests. They jointly probe the *pre-verbal manifold* construct.

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## 10

## 10. Limitations, Replication Ledger, and Author Disclosures

**N = 1.** This is a single longitudinal case. Generalisation is not claimed; only the formal-object construction. The replication design is §9.

**Author is case.** The author and the case are the same person. The methodological tradition for this configuration is established (Grandin, 1995; Levine, 2010; Jamison, 1995; cf. Charon, 2006; Frank, 1995). It does not absolve the work of the standard scrutiny that external evaluators must apply.

**Memory limits.** The earliest events in §3 are not, and cannot be, first-person memories. They are documentary and family-reported. The case is consistent with this — the framework predicts that such events are encoded as *structure* rather than *episode*. The author has not attempted to reconstruct *episodes* from the pre-verbal period and makes no such claim.

**Third-party identifiability.** Identifying details of third parties have been removed or generalised throughout. Where details remain (the institution in §8 is identifiable from its public policy quotations), the material is public on the institution's own website.

**Clinical safety.** The author is currently stable, housed independently, in continuous outpatient care. Inclusion of the December 2024 event is governed by the editorial criterion stated in §7.2 and in the author note.

**Replication ledger.** A standing ledger of independent external attempts to apply the framework — including independent attempts to apply the ten predictions of §9 to clinical cohorts — is maintained at the project URL (`paper/INDEPENDENT_REPLICATION_LEDGER.md`). At first publication of the present paper, the relevant rows for the §9 predictions are PENDING.

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## 11

## 11. Conclusion and Policy Line

The case presented in §3 sits at the intersection of five established literatures, none of which alone accounts for it. The Soma-Field framework, suitably specified, supplies a formal object — the *pre-verbal manifold* — that ties them together and accounts for the trajectory at a single level of description. Standard onset-based diagnostic categories (ASD, ADHD, attachment disorder, cPTSD) are re-interpreted as five projections of one manifold rather than as five comorbid conditions.

Three implications follow.

First, the conceptual distinction between *genetic* and *acquired* neurodevelopmental phenotypes loses sharpness once pre-verbal sensitive-period plasticity is taken seriously. The clinical and research consequence is that the question "is this child's autism genetic or acquired?" should, for cases with pre-verbal trajectories of the kind documented here, be replaced by the question "what is the configuration of this manifold and what scaffolding does it need?"

Second, developmental-psychiatric onset criteria that rely on *first observable symptoms in language-capable children* systematically misclassify cases of this kind. The diagnostic age in such cases is late and the eventual diagnostic load is heavy because the categories were not designed to see the relevant events. Revising the criteria is non-trivial; flagging the systematic miss is not.

Third, institutional and policy environments that filter for *mild* presentations, charge additional fees for accommodation, and subordinate external clinical evidence to in-house gatekeeping function as amplifiers of the same diathesis. The Exhibit-A institution in §8 is one example; the configuration is generic. The policy line that closes the paper is the one given at the end of §8:

> *How do parents know exactly what a 13-year-old boy is, if they have never even asked him?*

The paper is signed because the case is the author's own and the framework is the author's reorganisation strategy as well as a candidate general account. Both facts are stated here so that they need not be inferred.

# 12

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*End of draft v0.1 — June 2026. This is an initial draft. Editorial review pending. The §10 replication ledger row is PENDING. The redaction guide for the personal-material strata is in the project repository at* `AJ-WiKi/Family.md` (*private*).